
Day 18 of 42. *Can we stop?*

BED 7

Streptococcal aortic-valve
endocarditis

Afebrile · CRP falling

Illustrative patient

1	2	3	4	5	6	7
8	9	10	11	12	13	14
15	16	17	18	19	20	21
22	23	24	25	26	27	28
29	30	31	32	33	34	35
36	37	38	39	40	41	42

Six weeks comes mostly from 1950s observations

1950s

Lower-dose, single-drug
era

per POET-II investigators

2019

POET I: oral step-down

2026

POET-II: how long?

Only stable patients were randomized

- ☑ Afebrile for 2 days
- ☑ CRP < 50% of peak and < 40 mg/L
- ☑ WBC < $10 \times 10^9/\text{L}$
- ☑ Procalcitonin < 0.5 ng/mL
- ☑ No progression on TEE
- ☑ No further surgery planned



Randomized

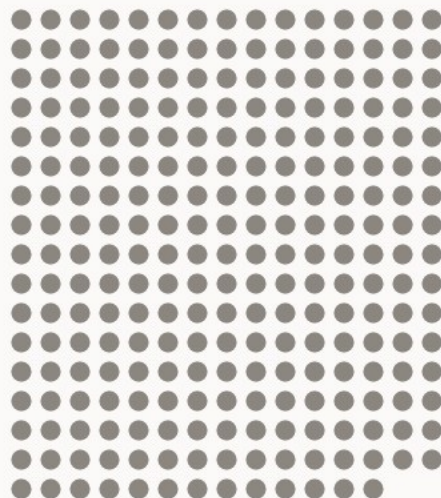
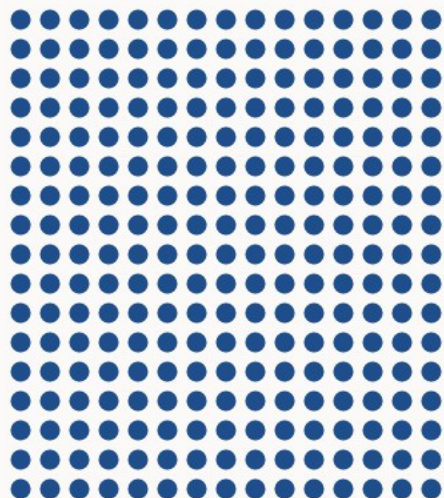
508 stable patients: stop now, or continue

255

Tailored – stop now

253

Standard – continue to 4–6 weeks



Open-label

13 centres

Mean age 70

25% women

Streptococci 57%

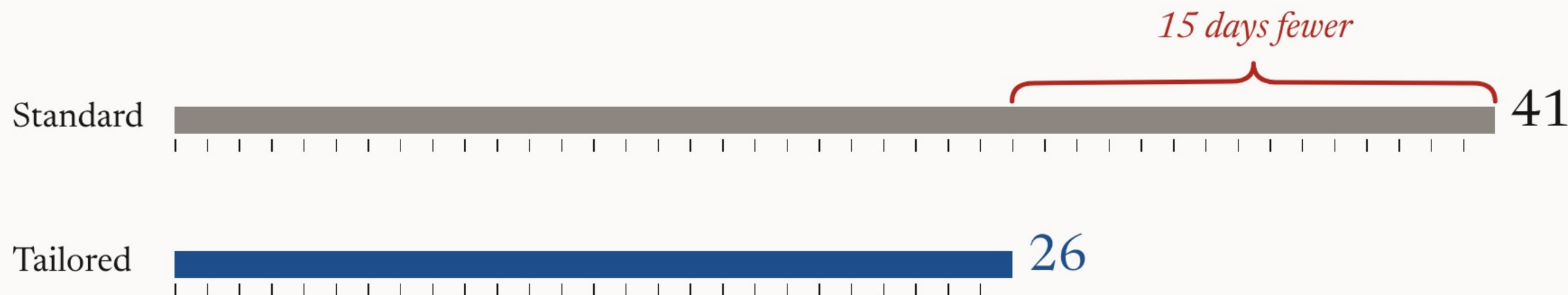
E. faecalis 19%



S. aureus 24%

Tailored therapy cut antibiotics by 15 days

–36.6%



Medians; 36.6% calculated. Illustrative calendar.

13 more days alive without antibiotics

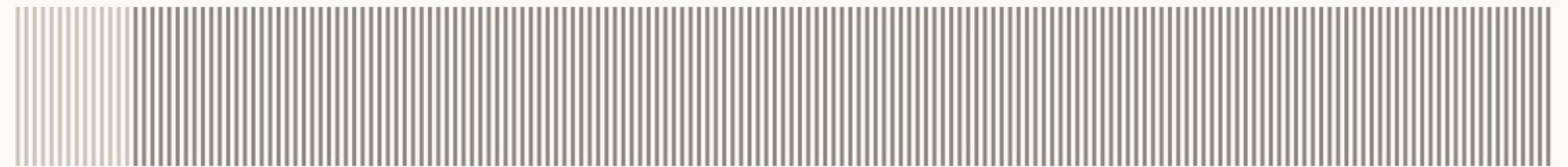
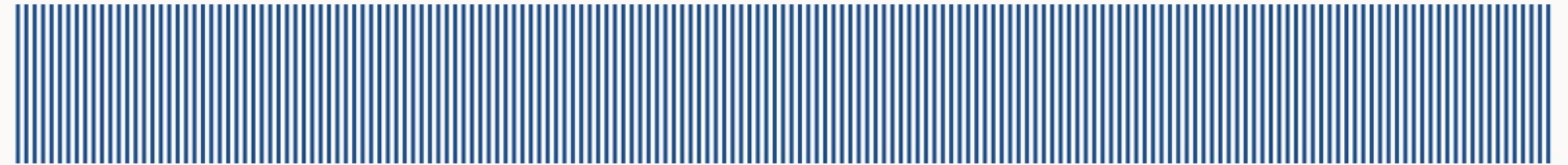
TAILORED

183

STANDARD

169

WITHIN SIX MONTHS



95% CI 12–13 · $P < 0.001$

Medians

Safety: noninferior at six months

TAILORED



8.2%

STANDARD

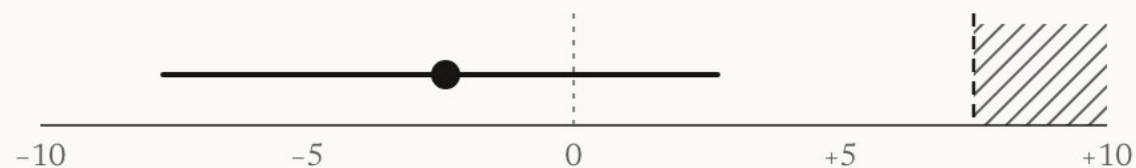


10.7%

✓ Noninferiority met

DIFFERENCE

MARGIN 7.5



Death

1.18 (0.53–2.63)

Unplanned surgery

0.49 (0.17–1.45)

Embolic events

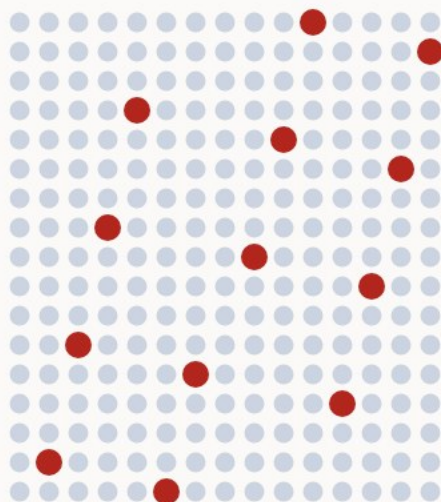
0.75 (0.26–2.15)



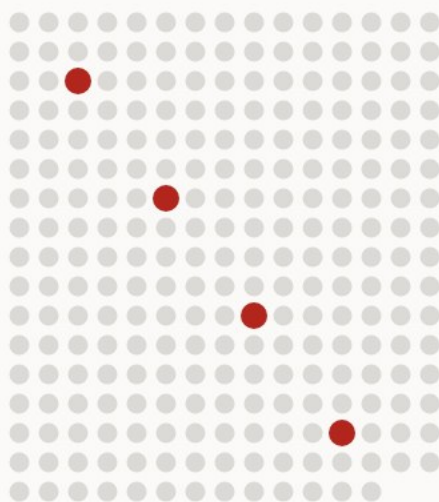
Wide intervals: few events

Relapse was more frequent: 5.1% vs 1.6%

TAILORED



STANDARD



13 *of 255*

4 *of 253*

$P = 0.04$

median days to relapse



Mostly uncomplicated; antibiotics restarted.

E. faecalis relapsed most

RELAPSE BY PATHOGEN

E. faecalis



6.4%

S. aureus



3.3%

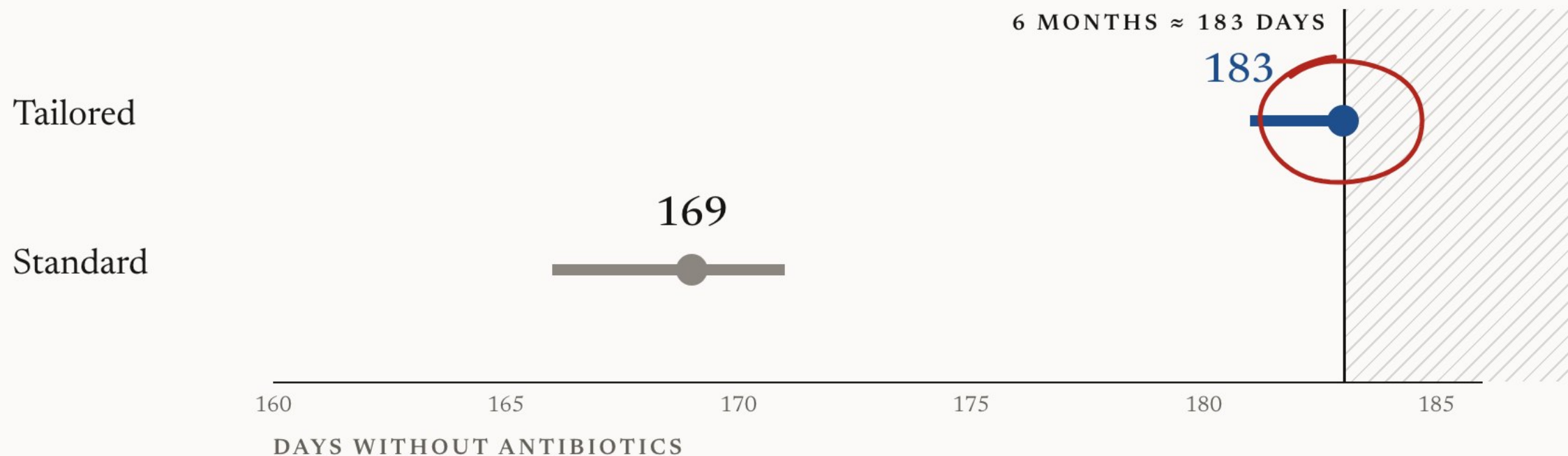
Streptococci



2.4%

Subgroup: hypothesis-generating

The primary endpoint is partly built in



Original primary (2020) included relapse

Real tests: safety, relapse. *(my reading)*

Five reasons for caution

- 1 Open-label
- 2 Selected stable patients – about half eligible
- 3 Specialized centres
- 4 Three pathogens only
- 5 Strict criteria, incl. TEE and procalcitonin

My reading – not a guideline

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■ All criteria met + strep or *S. aureus*:
discuss stopping

▨ *E. faecalis*: be cautious

□ Close follow-up in month one

Would you stop?

BED 7

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All six criteria met • streptococcus

- ☐ Stop
- ☐ Continue

What would change your mind?

Treat the response, not the calendar.

Questions?

Components of the safety endpoint

SIX MONTHS	TAILORED	STANDARD	HR (95% CI)
Death	5.1%	4.3%	1.18 (0.53–2.63)
Unplanned cardiac surgery	2%	4%	0.49 (0.17–1.45)
Symptomatic embolic events	2.4%	3.2%	0.75 (0.26–2.15)
Composite (primary safety)	21 (8.2%)	27 (10.7%)	Noninferiority met

Absolute difference –2.4 percentage points (95% CI – 7.7 to 2.7); noninferiority margin 7.5 percentage points.

Composite, difference and margin: NEJM abstract. Components as reported by TCTMD, 28 Aug 2026.

Stabilization criteria – all required

Temperature < 38 °C (100.4 °F) for at least 2 days

CRP < 50% of peak and < 40 mg/L

WBC < $10 \times 10^9/L$

Procalcitonin < 0.5 ng/mL

No progression on TEE within 48 h

No further cardiac surgery planned

Before randomization: at least 2–4 weeks of antibiotics

2–4 weeks before randomization: NEJM abstract. Criteria as reported by TCTMD and ACC – full definitions in the NEJM supplement.

Pathogen subgroups

PATHOGEN	SHARE OF TRIAL	RELAPSE	DIRECTION
Streptococci	57.3%	2.4%	Favoured tailored
<i>S. aureus</i>	24.2%	3.3%	Favoured tailored
<i>E. faecalis</i>	18.5%	6.4%	Favoured standard

Consistent across native vs prosthetic valves and medical vs surgical treatment. Adding relapse to the safety endpoint, or excluding extracardiac infections: unchanged.

Subgroups are hypothesis-generating. As reported by TCTMD.

Registered vs published outcome

	DESIGN PAPER 2020 · REGISTRY 2021	PUBLISHED 2026
Primary	One safety composite: death, unplanned surgery, embolism, relapse of bacteremia	Efficacy: days alive without antibiotics + safety: death, unplanned surgery, embolism
Relapse	Inside the primary composite	Key secondary: 5.1% vs 1.6%
Planned size	750 (registry, Dec 2021)	508 randomized

The change happened while the open-label trial was recruiting; when and why is not stated in the sources I could read. Authors report that adding relapse back to the safety endpoint left the result unchanged (TCTMD).

Østergaard L, et al. Am Heart J 2020;227:40–46 (PMID 32673830) · ClinicalTrials.gov NCT05144399 (duplicate record, 3 Dec 2021) · NCT03851575 (current).

ESC 2023 guideline – durations

Streptococci, native valve: 4 weeks (2 weeks with gentamicin in selected uncomplicated cases)

Streptococci, prosthetic valve: 6 weeks

S. aureus (MSSA): native valve 4–6 weeks; prosthetic at least 6 weeks

E. faecalis: amoxicillin + ceftriaxone, 6 weeks

Oral step-down after at least 10 days IV in stable patients: class IIa

Delgado V, et al. Eur Heart J 2023;44:3948–4042 (PMID 37622656). Read from an early-online copy – confirm wording in the final version.

POET I – the route question

Iversen K, et al. NEJM 2019;380:415–424 · PMID 30152252

Stable left-sided endocarditis

Partial oral step-down was noninferior to continued IV

POET I changed the route. POET-II changes the duration.

Background as cited in the ESC Congress 2026 press release.

Who is eligible?

About half of left-sided IE patients could be eligible (investigators' estimate)

The three pathogens cause about 70% of IE overall (per authors)

ESC discussant G. Habib: personalized duration – in selected patients, in specialized endocarditis centres

He asked for more detail on stabilization criteria, and caution in *E. faecalis*

ESC press release; TCTMD, 28 Aug 2026.

Funding and registration

Investigator-initiated; funded by Sygeforsikringen “danmark” and others

ClinicalTrials.gov NCT03851575

DOI 10.1056/NEJMoa2607887 · PMID 42663314

ESC Congress 2026, Hot Line 1, Munich, 28 Aug 2026

PI: Prof. Henning Bundgaard, Rigshospitalet, Copenhagen

Bundgaard H, et al. N Engl J Med, published online 28 Aug 2026 (abstract, PubMed).